

[Current Alzheimer's Disease Therapeutics Landscape vs. CXXC5–Dvl Platform]

Category	Current Alzheimer' Disease Drugs	CXXC5–Dvl Platform (CK Regeon)
Target	Amyloid, p-Tau	CXXC5–Dvl PPI
MOA	Amyloid beta inactivation by antibodies	Restores endogenous regenerative capacity by reactivating Wnt/ β -catenin signaling, leading to the upregulation of key genes involved in neuronal structure, function, and tissue regeneration (e.g., <i>Neurogenin</i> , <i>ADAM10</i> , <i>NeuroD1</i> , <i>CACNA1G</i> , <i>GLP-1</i> , <i>LGR5</i>).
Efficacies	Mild AD patients, Early stage patients, Effect size is small (Symptom control) * Approved mAbs have shown about 27-35% slowing of decline in early AD, but no true recovery and ongoing debate on clinical meaningfulness.	Regenerative, disease-modifying therapeutics targeting root-cause correction, with additional reduction of β -amyloid and p-Tau levels via GSK3 β modulation
Candidate-Administration	Antibody-Injection	Small molecules-Oral
Side effects	Chronic-usage could cause brain edema/hemorrhage, brain bleeds, stroke-like symptoms	Targeting the disease-mediating negative regulatory protein CXXC5 (release negative regulatory mechanism involving AD)—rather than directly activating the WNT pathway—provides a safer approach to modulating this pathway, as confirmed by multiple preclinical studies.
New Target Requirement	Targeting amyloid β is limited due to over-reliance on amyloid hypothesis.	First-in-class upstream target releasing the disease-associated CXXC5 “brake,” restoring WNT pathway gene expression to drive cellular function and tissue regeneration.